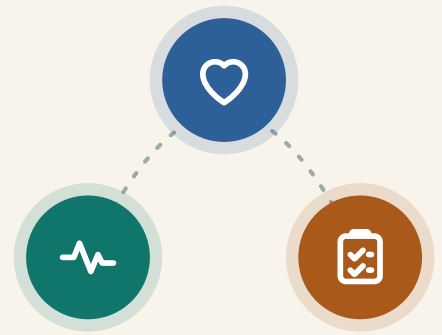


ACE-T

The first steps after a positive delirium screen



Aim for four hours

Complete, start or escalate the actions that apply within four hours of the positive screen. **Act on urgent concerns immediately.** Work on the three domains in parallel, within your role and local policy.

AC

Acute Triggers

What might be causing or worsening the delirium?

- ☐ Check observations
- ☐ Check capillary blood glucose
- ☐ Check that routine blood tests have been sent
- ☐ Assess for urinary retention and constipation
- ☐ Assess hydration status
- ☐ Assess nutrition status
- ☐ Start or review an infection work-up, as appropriate
- ☐ Assess pain
- ☐ Check whether a medication review has been completed

E

Patient Experience

How is the person feeling, and what would help?

- ☐ Check for distress
- ☐ Note any agitation
- ☐ Provide reassurance and reorientation
- ☐ Consider the patient's environment
- ☐ Make sure glasses, hearing aids and other communication aids are available
- ☐ Consider involving relatives, friends or carers

T

Treatment

What does the person need now, and who needs to know?

- ☐ Consider falls risk
- ☐ Consider whether fluids, oxygen or other immediate supportive measures are needed
- ☐ Document "delirium" or "? delirium" and the 4AT score in the clinical record
- ☐ Communicate the screening score and relevant findings to the clinical team
- ☐ Inform the family, where appropriate
- ☐ Offer the delirium information leaflet to the patient and family, where appropriate
- ☐ Discuss the treatment plan with the multidisciplinary team

Detect

Positive 4AT (4 or more), positive local screen, or suspected delirium



Respond with ACE-T

Nursing actions in the first four hours, alongside medical assessment



Shared plan

Delirium documented, team and family informed, plan agreed